

February 20, 2026

Representative Emanuel “Chris” Welch
Speaker of the House
300 Capitol Building
Springfield, IL 62706

Representative Robyn Gabel
Majority Leader of the House
300 Capitol Building
Springfield, IL 62706

Representative Tony M. McCombie
Republican Leader of the House
316 Capitol Building
Springfield, IL 62706

Representative Mary Beth Canty
280-S Stratton Office Building
Springfield, IL 62706

RE: Letter in Opposition to the Illinois Protect Health Data Privacy Act (HB 3494)

Dear Speaker Welch, Majority Leader Gabel, Republican Leader McCombie, and Representative Canty:

On behalf of the advertising industry, we write to oppose the Illinois Protect Health Data Privacy Act, HB 3494.¹ If enacted, the bill would hinder individuals in Illinois from accessing services and burden the operation of many businesses in the state. Our organizations support the enactment of meaningful privacy protections for Illinoisans. However, as presently drafted, HB 3494 would have far-reaching, unintended, and unfavorable consequences for individuals in Illinois and the business community alike. We therefore strongly encourage you to decline to advance the bill any further in the legislative process.

As the nation’s leading advertising and marketing trade associations, we collectively represent thousands of companies across the country. These companies range from small businesses to household brands, advertising agencies, and technology providers. Our combined membership includes more than 2,000 companies that power the commercial Internet, which accounted for nearly 20 percent of total U.S. gross domestic product (“GDP”) in 2024.² By one estimate, approximately 18.8% of Illinois jobs in 2024 were related to the ad-subsidized Internet, a share projected to increase to 20.8% by 2029.³ Our group has more than a decade’s worth of hands-on experience it can bring to bear on matters related to consumer privacy and controls. We would welcome the opportunity to engage with you further on the points we discuss in this letter.

I. HB 3494’s Definitions of “Health Data” and “Individual” are Overly Broad

HB 3494’s terms, coupled with its overly broad definition of “health data,” could unintentionally impede Illinoisans from receiving useful and relevant information about products and services they may desire. As defined, the term “health data” would include *any information*

¹ Illinois HB 3494 (2026 Sess.), located [here](#) (hereinafter, “HB 3494”).

² S&P Global, THE ECONOMIC IMPACT OF ADVERTISING ON THE US ECONOMY, 2024-2029 at 4 (Aug. 2025), located at https://theadcoalition.com/wp-content/uploads/2025/08/TAC_SP-Global-Final-Report_August-2025.pdf.

³ *Id.* at 15-16.

that could possibly be related—however tangentially—to the health of an individual.⁴ The definition could be interpreted to include basic data points, such as the fact that an individual purchased non-prescription shampoo for individuals with dry hair at a local grocer, attended a fitness class, or signed up to receive promotional notices about specific clothing or footwear restocks. None of this information is inherently related to health, but HB 3494’s broad definition of “health data” could sweep such information into its ambit.

The House of Representatives (“House”) should decline to advance HB 3494 any further in the legislative process, or, at the very least, should update the bill to narrow the definition of the term “health data.” We recommend that the House align the bill’s definition of “health data” with the majority of states that have passed legislation regulating consumer health data, such as the definitions of the term in the Connecticut, Maryland, and Nevada laws.⁵ In those state laws, “consumer health data” is personal information that is linked or reasonably capable of being linked to a consumer and that “*is used to identify*” the past, present, or future health status of the consumer. This definition will help ensure that the term is cabined to information actually used to identify a consumer’s health status. This ensures that less sensitive data points are not unintentionally swept into the definition of the term.

HB 3494 should also be amended to clarify that “individual” refers exclusively to Illinois residents. We recommend refining the existing broad term defining a “individual” to remove the presumption that a person is an Illinois resident when the person is located in Illinois when health data is collected.⁶ This update will help clarify that the bill applies exclusively to “health data” associated with Illinoisans and would help harmonize the bill’s terms with laws in other states.

II. The Bill’s Consent and Data Minimization Requirements Would Cause Frustration for Individuals in Illinois and Impede Innovation

HB 3494 would require a regulated entity that processes health data to obtain consent from the individual to which the health data pertains.⁷ Otherwise, the bill would permit health data processing only if strictly necessary to provide a product or service the individual specifically requested from the regulated entity.⁸ If a regulated entity seeks to “sell” health data, the bill would require signed, written authorization from the individual.⁹ Made more onerous due to HB 3494’s broad definitions, as discussed in Section I above, these consent requirements would inundate individuals with an overwhelming number of consent requests for basic data processing activities. The bill is consequently likely to result in significant consent fatigue for individuals in Illinois instead of providing meaningful privacy protections. Moreover, the signed written authorization requirement would introduce considerable friction into Illinoisians’ experiences with companies without providing commensurate benefits. The bill is also likely to severely degrade innovation and development in health-related products and services, as

⁴ HB 3494 § 5.

⁵ See Conn. Gen. Stat. § 42-515(9); Md. Code Ann., Com. Law § 14-4601(I); Nev. Rev. Stat. § 603A.430.

⁶ HB 3494 § 5.

⁷ *Id.* § 20(a).

⁸ *Id.*

⁹ *Id.* § 25(a).

processing health data would be permissible only in the context of a product or service the individual already has requested.

Additionally, the bill defines “health data” to include precise location information that could reasonably be used to determine an individual’s attempt to acquire or receive health services or supplies.¹⁰ The bill provides no information regarding how a regulated entity can evaluate if precise geolocation information can “reasonably be used to determine” an attempt to acquire health services or supplies. The bill’s restrictions on health data could consequently chill advertising and marketing efforts as well as important services that depend on precise location data to function. HB 3494’s detrimental—and likely unintentional—consequences would hinder individuals in Illinois from receiving significant benefits associated with routine and essential data practices while simultaneously placing overly burdensome requirements on regulated entities that process any information that might or might not even vaguely be related to health.

III. HB 3494 Would Diverge Sharply from Existing Privacy Laws by Requiring Regulated Entities to Disclose Names of Third-Party Partners

HB 3494 would significantly deviate from existing state privacy laws in that it would require regulated entities to disclose the names of their specific third-party partners as part of required disclosures in the health data privacy policy.¹¹ The overwhelming majority of state privacy laws require companies to disclose the *categories* of third parties to whom they transfer personal data rather than the specific names of such third parties themselves.

Requiring disclosure of the names of third-party entities would be operationally burdensome, as potential regulated entities change business partners frequently, and companies regularly merge with others and change names. For instance, a regulated entity may engage in a data exchange with a new business-customer on the next day after it posts their health data privacy policy to their website. This requirement would either force the regulated entity to refrain from engaging in commerce with the new business-customer until its disclosures are updated, or risk violating the law. This is an unreasonable restraint. From an operational standpoint, constantly updating lists of all third-party partners a regulated entity works with would take significant resources and time away from efforts to responsibly process information about individuals.

Even international privacy standards like the European Union’s General Data Protection Regulation (“GDPR”) do not require burdensome disclosures of specific third parties in response to data subject access requests, according to the text of the law. Mandating that companies disclose the names of their third-party partners could obligate companies to abridge confidentiality clauses they maintain in their contracts with partners and expose proprietary business information to their competitors. Finally, the individual benefit that would accrue from receipt of a list of third-party partners to whom a regulated entity discloses data would be minimal at best. For these reasons, we encourage you to reconsider this onerous requirement, which severely diverges from the approach to required disclosures taken in existing state laws.

¹⁰ *Id.* § 5.

¹¹ *Id.* § 15(a)(6).

IV. Requiring Regulated Entities to Pass Individual Rights Requests to Third Parties Could Conflict with Individual Preferences

HB 3494 would require regulated entities to pass along deletion requests to service providers, contractors, *and third parties* and would require any party that receives notice of such a request to delete all health data associated with the individual.¹² HB 3494 should not require regulated entities to flow deletion requests through to third parties because such actions may not align with individuals' wishes. An individual may desire, for instance, to delete the health data maintained by a certain regulated entity but may not want other regulated entities—who would be “third parties” under HB 3494’s terms—to similarly be required to delete health data. Such a requirement could impact products and services the individual wishes and expects to receive, as a deletion request served on one entity would be required to be cast to all other entities in the marketplace. The individual may not intend or want to serve a deletion request on regulated entities the individual knows, trusts, and wishes to continue to receive messaging from. Requiring deletion requests to be passed to other entities in the marketplace would create detrimental results the individual did not intend, desire, or expect.

V. A Private Right of Action Is an Inappropriate Form of Enforcement for Privacy Legislation

As presently drafted, HB 3494 allows an individual to bring a lawsuit for an alleged violation of the bill.¹³ HB 3494 should be updated to clarify that it does not create a private right of action under any law. We strongly believe private rights of action should have no place in privacy legislation. Instead, enforcement should be vested with the Attorney General (“AG”) alone because such an enforcement structure would lead to stronger outcomes for individuals in Illinois while better enabling businesses to allocate resources to developing processes, procedures, and plans to facilitate compliance with new data privacy requirements. AG enforcement, instead of a private right of action, is in the best interests of individuals and businesses alike.

The private right of action in HB 3494 would create a complex and flawed compliance system without tangible privacy benefits for individuals. Allowing private actions would flood Illinois’s courts with frivolous lawsuits driven by opportunistic trial lawyers searching for technical violations, rather than focusing on actual individual harm.¹⁴ Private right of action provisions are completely divorced from any connection to actual individual harm and provide individuals little by way of protection from detrimental data practices.

¹² *Id.* § 50(b)(2), (d).

¹³ *Id.* § 75.

¹⁴ A select few attorneys benefit disproportionately from private right of action enforcement mechanisms in a way that dwarfs the benefits that accrue to the consumers who are the basis for the claims. For example, a study of 3,121 private actions under the Telephone Consumer Protection Act (“TCPA”) showed that approximately 60 percent of TCPA lawsuits were brought by just forty-four law firms. Amounts paid out to consumers under such lawsuits proved to be insignificant, as only 4 to 8 percent of eligible claim members made themselves available for compensation from the settlement funds. U.S. Chamber Institute for Legal Reform, *TCPA Litigation Sprawl* at 2, 4, 11-15 (Aug. 2017), located [here](#).

Additionally, a private right of action would have a chilling effect on the state's economy by creating the threat of steep penalties for companies that are good actors but inadvertently fail to conform to technical provisions of law. Private litigant enforcement provisions and related potential penalties for violations represent an overly punitive scheme that do not effectively address privacy concerns or deter undesired business conduct. They expose businesses to extraordinary and potentially enterprise-threatening costs for technical violations of law rather than drive systemic and helpful changes to business practices. A private right of action would also encumber businesses' attempts to innovate by threatening companies with expensive litigation costs, especially if those companies are visionaries striving to develop transformative new technologies. The threat of an expensive lawsuit may force smaller companies to agree to settle claims against them, even if they are convinced the claims are without merit.¹⁵

Beyond the staggering cost to Illinois businesses, the resulting snarl of litigation could create a chaotic and inconsistent enforcement framework with conflicting requirements based on differing court outcomes. Overall, a private right of action would serve as a windfall to the plaintiff's bar without focusing on the business practices that actually harm individuals. We therefore encourage the House to clarify that HB 3494 does not create a private right of action under any law and vests enforcement authority with the AG alone.

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¹⁵ For instance, in the early 2000s, private actions under California's Unfair Competition Law ("UCL") "launched an unending attack on businesses all over the state." American Tort Reform Foundation, *State Consumer Protection Laws Unhinged: It's Time to Restore Sanity to the Litigation* at 8 (2003), located [here](#). Consumers brought suits against homebuilders for abbreviating "APR" instead of spelling out "Annual Percentage Rate" in advertisements and sued travel agents for not posting their phone numbers on websites, in addition to initiating myriad other frivolous lawsuits. These lawsuits disproportionately impacted small businesses, ultimately resulting in citizens voting to pass Proposition 64 in 2004 to stem the abuse of the state's broad private right of action under the UCL. *Id.*



We and our members support protecting individuals' privacy. We believe, however, that HB 3494 takes an overly broad approach to the collection, use, and disclosure of any data that could possibly be related to or indicative of health. We therefore respectfully ask that you to reconsider the bill. We would also very much welcome the opportunity to engage with you further regarding an appropriate way to define "health data" so Illinoisans can enjoy robust protections without forfeiting the ability to receive benefits from the modern data-driven economy.

Thank you in advance for your consideration of this letter.

Sincerely,

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